

# Reading the Nail

*The consultation, common conditions, and knowing precisely where your practice ends*

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## Learning Objectives

- Conduct a full nail and surrounding-skin assessment before any service begins
- Identify the common nail conditions a practitioner will regularly encounter
- Recognise nail changes that may signal an underlying medical condition
- Distinguish conditions within scope from those requiring medical referral

The nail can be the first place a story shows

Nails are unusually honest. They record recent illness in a single horizontal groove, reveal nutrient deficiencies in their shape, and sometimes surface signs of conditions the client has not yet been diagnosed with at all. A careful assessment is not procedural caution alone — it is occasionally the moment a client is first gently pointed toward a diagnosis that matters.

The four-part nail assessment

1. **Visual scan.** Colour, shape, texture, discolouration, separation of the plate from the bed, and the condition of the surrounding skin and cuticle.
2. **Tactile check.** Gently assess for tenderness, warmth, or unusual thickness.
3. **Verbal history.** Recent illness, medication changes, injury, and any known conditions affecting circulation or immunity, covered further in Module Eight.
4. **Baseline comparison.** Using the colour-reading principles from Module Two, compare current presentation to the client's own established normal.

Conditions within a practitioner's scope

- **Mild hangnails and dry cuticles.** Manageable with gentle care and oiling, covered in Module Five.
- **Ordinary vertical ridging.** Age-related, cosmetic, no referral needed.
- **Mild, uninfected ingrown edges.** Careful, conservative filing and client education on proper at-home trimming.

#### Conditions that require referral

- **Onychomycosis (fungal nail infection).** Presents as thickened, yellowed, or crumbling nail, often starting at the free edge. Contagious and requires medical antifungal treatment — never treat cosmetically or attempt to "polish over."
- **Paronychia (infected cuticle/nail fold).** Redness, swelling, warmth, and pain around the nail fold, sometimes with visible pus, signals infection requiring medical attention, not a manicure.
- **Psoriatic nail changes.** Small pits across the plate, separation of the nail from the bed, or an oil-drop-like discolouration can indicate nail psoriasis, best managed jointly with a dermatologist.
- **Koilonychia (spoon-shaped nails).** A concave, spoon-like nail shape can indicate iron-deficiency anaemia and deserves a gentle referral conversation, not a cosmetic fix.
- **Clubbing.** A pronounced downward curving and bulbous swelling of the fingertip and nail can signal underlying cardiovascular or respiratory conditions and always warrants medical referral.

#### A Note from Practice

A client arrives for a routine manicure and mentions her nails have "gone a bit spoon-shaped lately, probably just from typing too much." A visual scan confirms a genuine concave curvature across several nails, consistent with koilonychia — a pattern with a real, common medical cause worth investigating, not a typing habit.

The correct response is not to proceed with polish and say nothing. It is to mention gently that this specific shape change is sometimes linked to iron levels, suggest she mention it to her doctor at her next visit, and proceed with a gentle, non-aggressive service today rather than one involving heavy filing or extensions on nails that may already be structurally compromised.

## Key Terms

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[object Object]

1

[object Object]

2

[object Object]

3

[object Object]

## Practical Activity · The Assessment Card

Perform a full four-part nail assessment on a partner or model. Complete a written consultation card recording your findings, including a note on whether anything observed would call for referral. **File this card; it is your first completed nail consultation record.**